جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : محمد صلاح راشد حسين
المحافظة : القاهره **الحي / المدينة :** مدينه نصر الاول **العنوان :** مدينه نصر
رقم البطاقة : 27709220103812 **رقم التليفون :** 01115879458 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 22-09-1977 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان الرئة
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان الرئة
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [6352] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: محمد صلاح راشد حسين
National ID: 27709220103812

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of lung cancer for Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


بطاقة تحقيق الشخصية
محمد
صلاح راشد حسن مسعود
٤٥ ش ابن النفيس
مدينة نصر اول - القاهره
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وسيط تامين هر
ذكر
مسلم
متزوج
البطاقة سارية حتى ٢٠٣١/٠٩/١١
١

Visit Number:	46826512338			Client Name:	
Patient Name:	Mohamed Salah Rashed			Referred by Prof:	Tarek Mohamed
Age / Sex:	48 year / Male			Exam Date:	26 Sep 2026
Patient ID:	2113352676	**Acc**	**2178790**	Printed:	27 Sep 2026 09:42:54 AM

PT

Whole Body PET/CT Examination

History and Clinical Data: -

Patient data: -

- **Male patient, 48 years old, Weight: 110 Kg, Blood Glucose level: 114 mg/dl**

Radiopharmaceutical: -

- F-18-FDG 6.0mCi injected I.V.

Clinical background: -

- **Right lung consolidation. Histopathology is inconclusive, atypical epithelial neoplasm.**

Purpose of the study: -

- **Suspecting lung cancer.**

TECHNIQUE: -

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- **Liver showed average SUV ~1.61 (max. SUV~3.59).**

REPORT: -

Review of the PET, CT and the fused images revealed-

I) **Chest:**

- Metabolically active (FDG avid) right upper lung lobe consolidative mass like lesion extending to the hilar region merging with the enlarged FDG avid right hilar LN, roughly measuring about 9.3x9.7 cm in maximum diameters and achieving max. SUV~8.61.
- Diffuse metabolically active bilateral pulmonary reticulo-nodular infiltrates.
- Few FDG avid right pleural plaques, measuring about 1x2.7 cm with SUVmax 3 associated with low-grade metabolically active mild pleural effusion.
- Minimal left pleural effusion. No FDG avid lesions.
- Multiple enlarged metabolically active (FDG avid) mediastinal pre-tracheal, right para-tracheal, retro-caval, prevascular, aorto-pulmonary, carinal, subcarinal, bilateral hilar, and right cardio-phrenic LNs, the most active is the subcarinal LN, measuring about 1.8 cm with SUVmax 7.62.

II) **Distant nodal:**

- Few enlarged metabolically active (FDG avid) lymph nodes are seen at:
  - Right supraclavicular group, measuring about 1.5 cm with SUVmax 6.7.
  - Upper abdominal right retro-crural, left gastric, celiac, porto-caval, retro-pancreatic and left para-aortic LNs, measuring about 2 cm with SUVmax 3.67.

III) **Liver:**

- Fatty liver showing few metabolically active (FDG avid) hepatic focal lesions, the most active located at segment VII, measuring 1.8 cm with SUVmax 6.82.
- **Skeleton:**
- Widespread FDG avid sclerotic osseous lesions are seen throughout the axial and appendicular skeleton, the most active is D8 lesion achieving SUVmax 5.64.
- No associated extra-osseous soft tissue component.

IV) **No other metabolically active (FDG avid) lesions detected all over the rest of the body.**

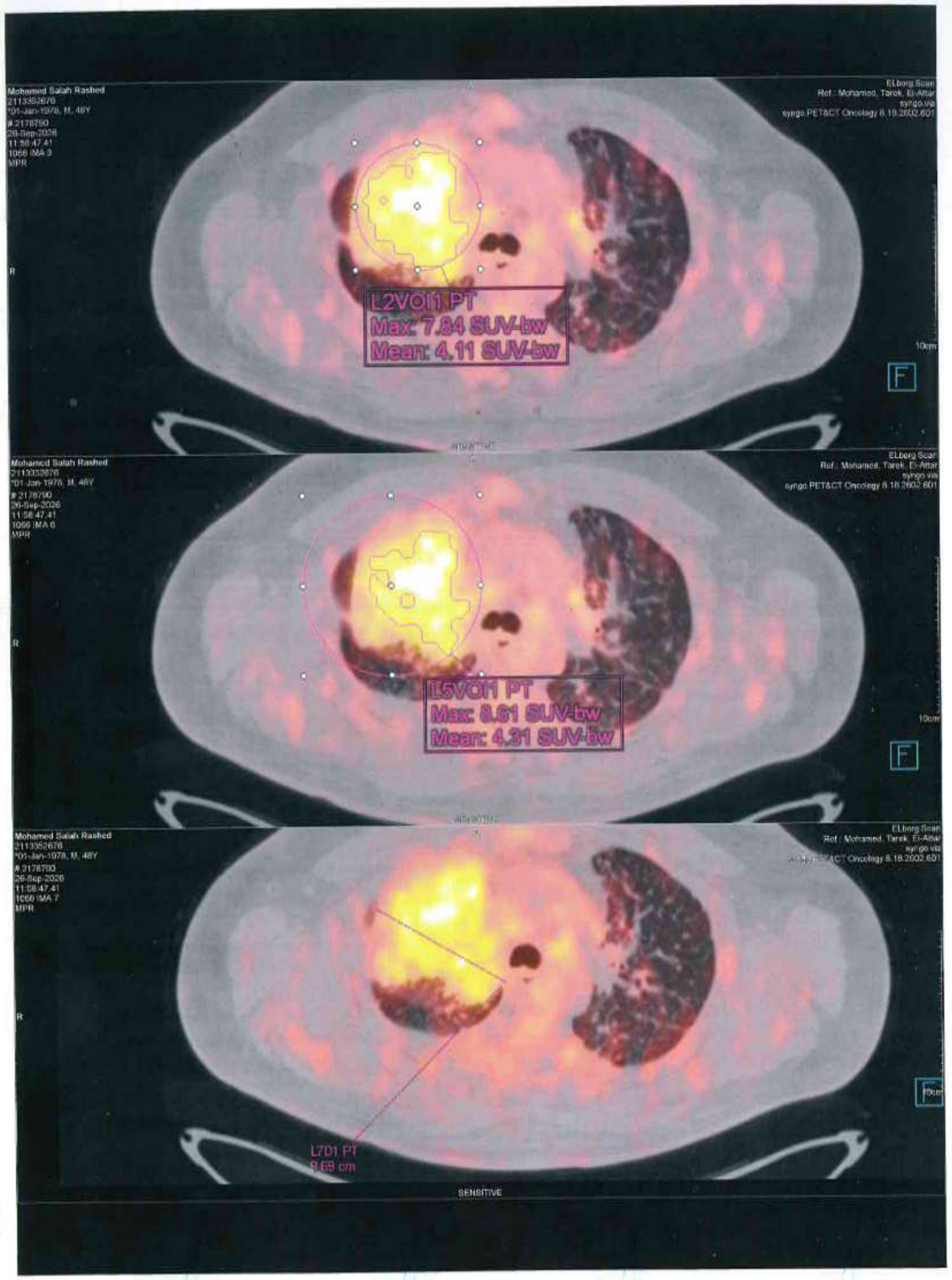
V) **The rest of the body:**

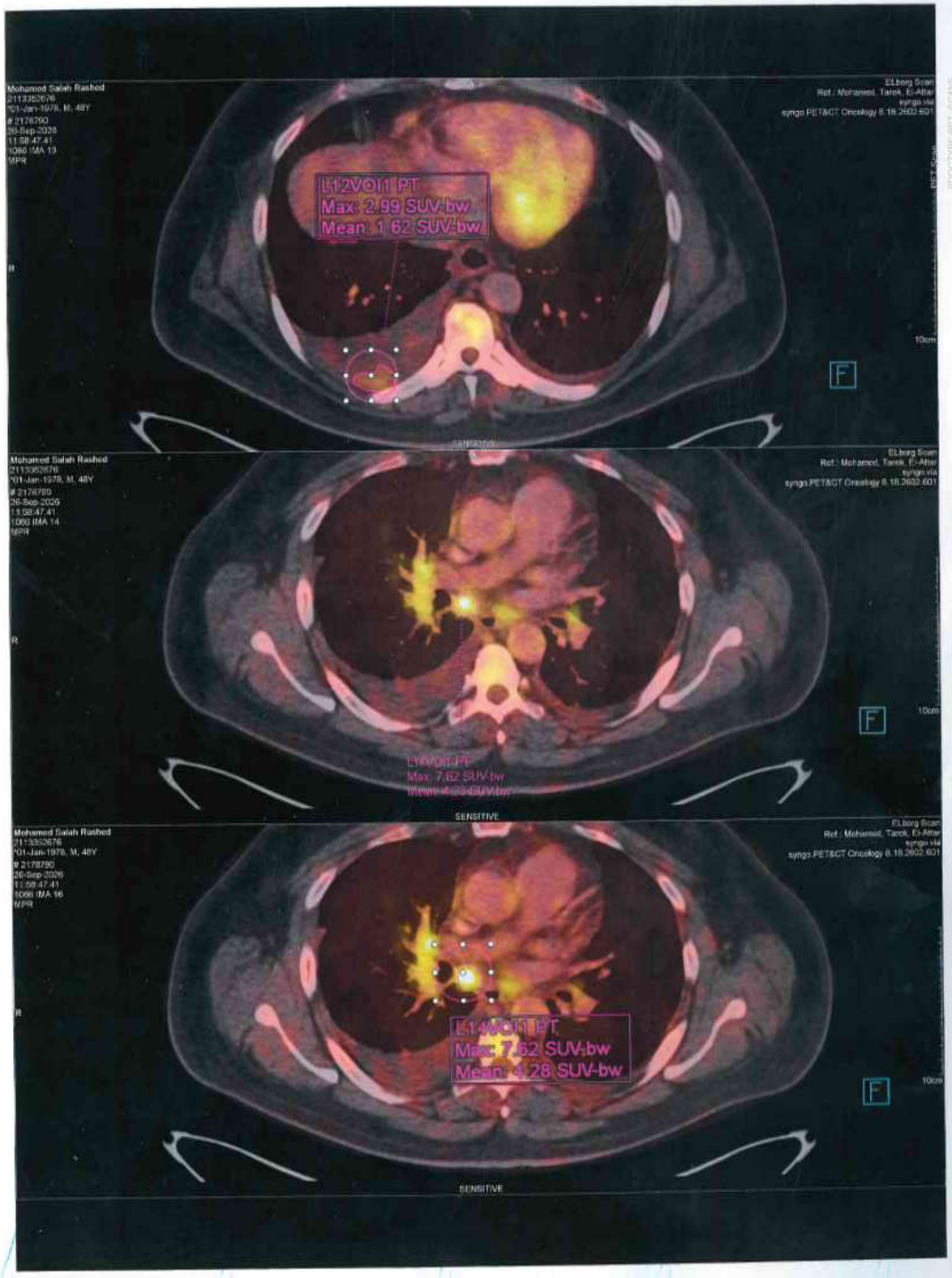
- **Brain**: No FDG-avid intracranial space occupying lesions detected.
- **Spleen**: average in size showing normal FDG uptake with no metabolically active FDG avid splenic focal lesions.

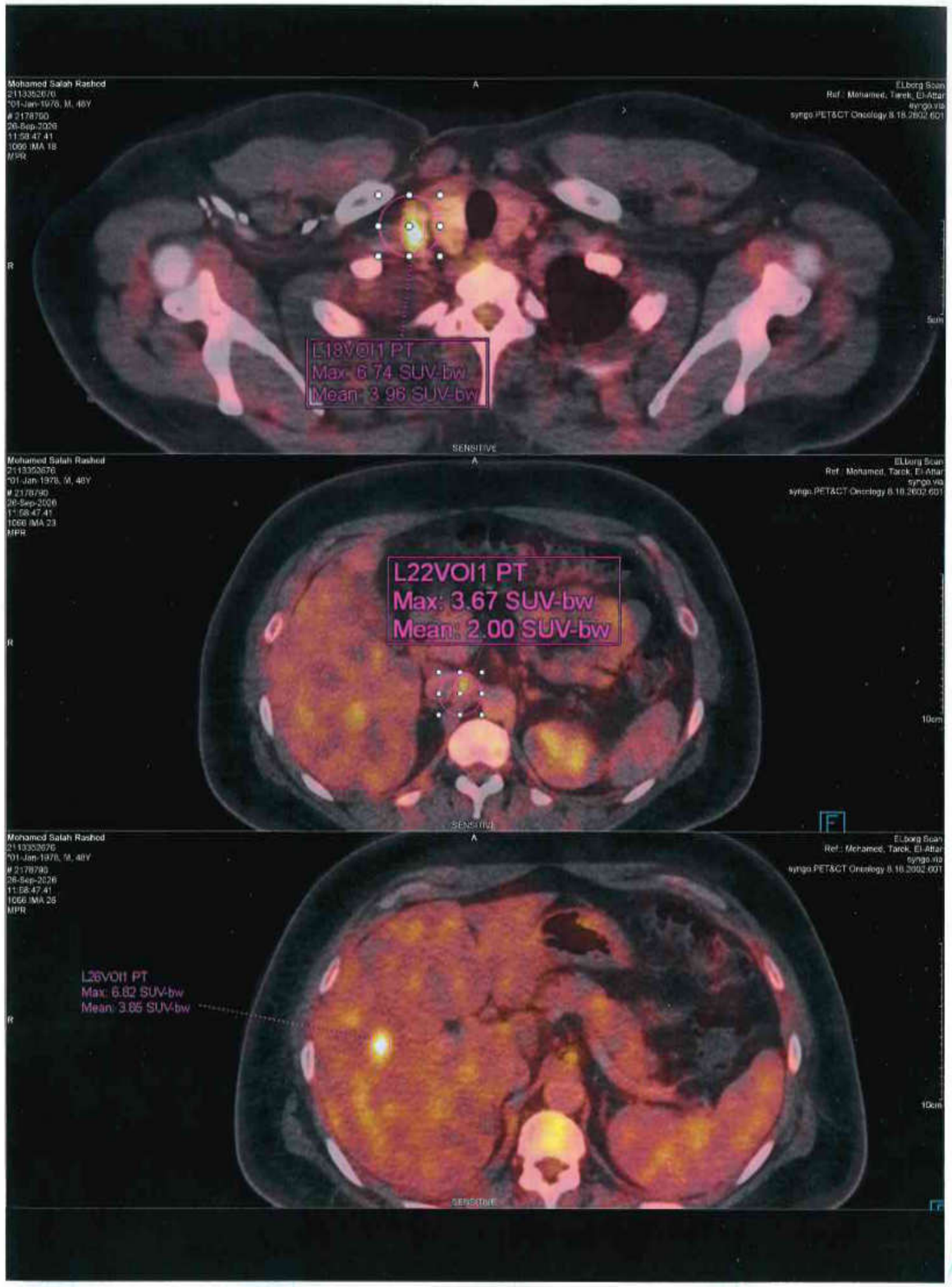
Side Findings:

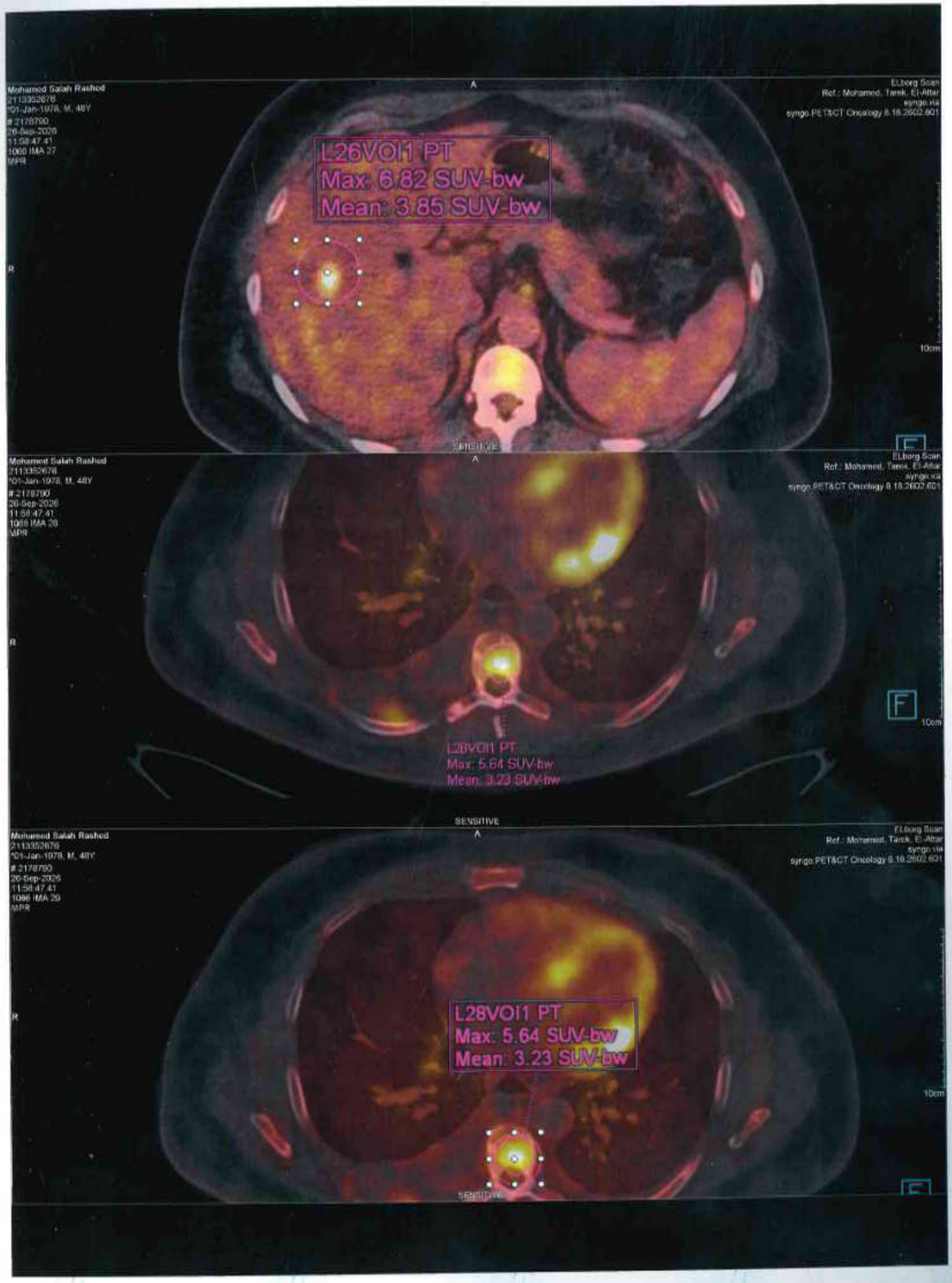
- Metabolically active right thyroid lobe nodule, measuring about 1.2x2.2 cm with SUVmax 3.5, for Doppler U/S correlation.

PET Sign [[DOCTYPE.PET_SCAN]]

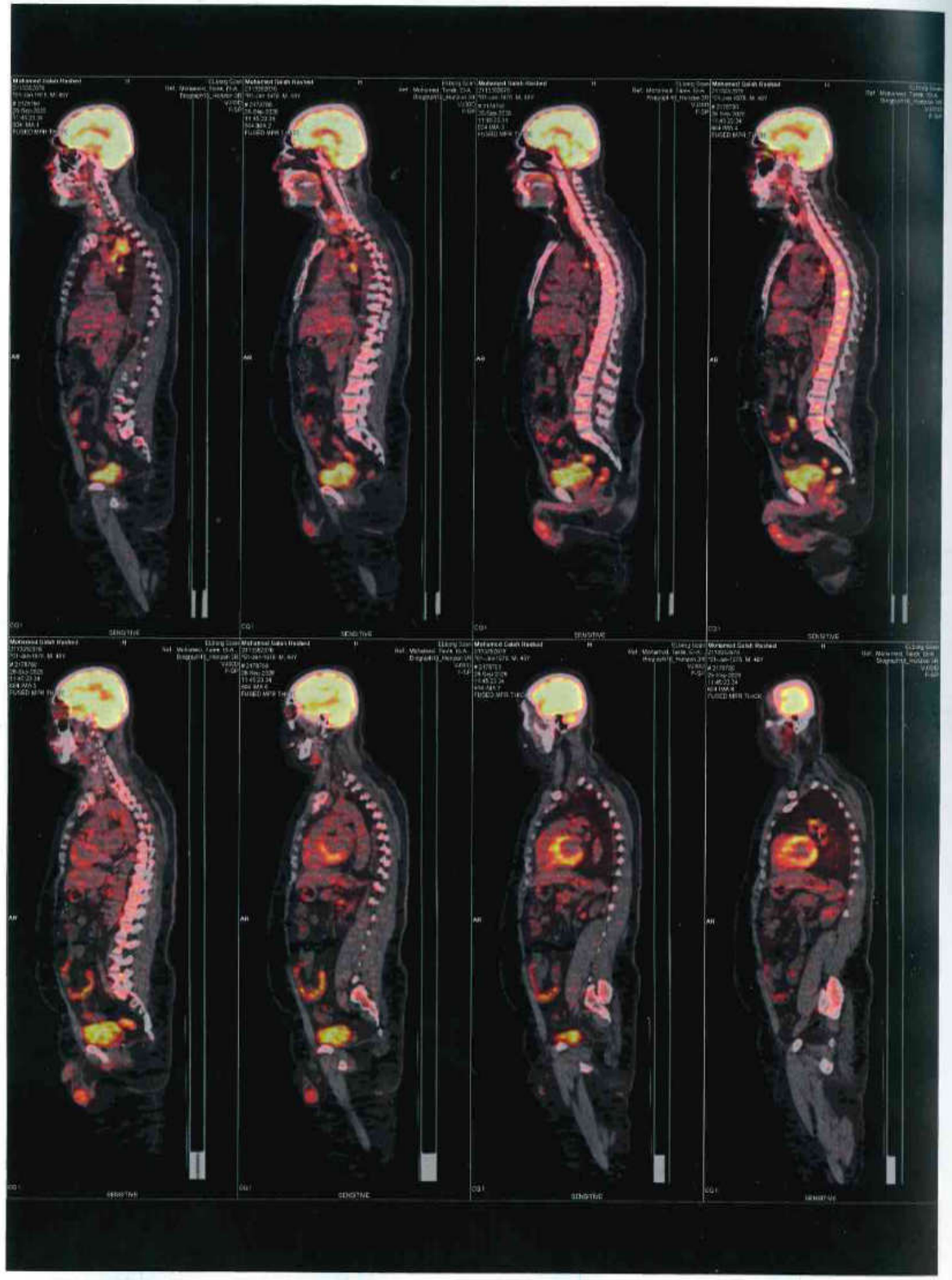

Mohamed Salah Rashed
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26-Sep-2026
11:58:47.41
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MPR
ELberg Scan
Ref.: Mohamed, Tarek, El-Attar
syngo.via
syngo.PET&CT Oncology 8.18.2602.601
L2VOI1 PT
Max: 7.84 SUV-bw
Mean: 4.11 SUV-bw
10cm
F
Mohamed Salah Rashed
2113352676
*01-Jan-1978, M, 48Y
2178790
26-Sep-2026
11:58:47.41
1066 IMA 6
MPR
ELberg Scan
Ref.: Mohamed, Tarek, El-Attar
syngo.via
syngo.PET&CT Oncology 8.18.2602.601
L5VOI1 PT
Max: 8.61 SUV-bw
Mean: 4.31 SUV-bw
10cm
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ELberg Scan
Ref.: Mohamed, Tarek, El-Attar
L7D1 PT
9.69 cm
SENSITIVE

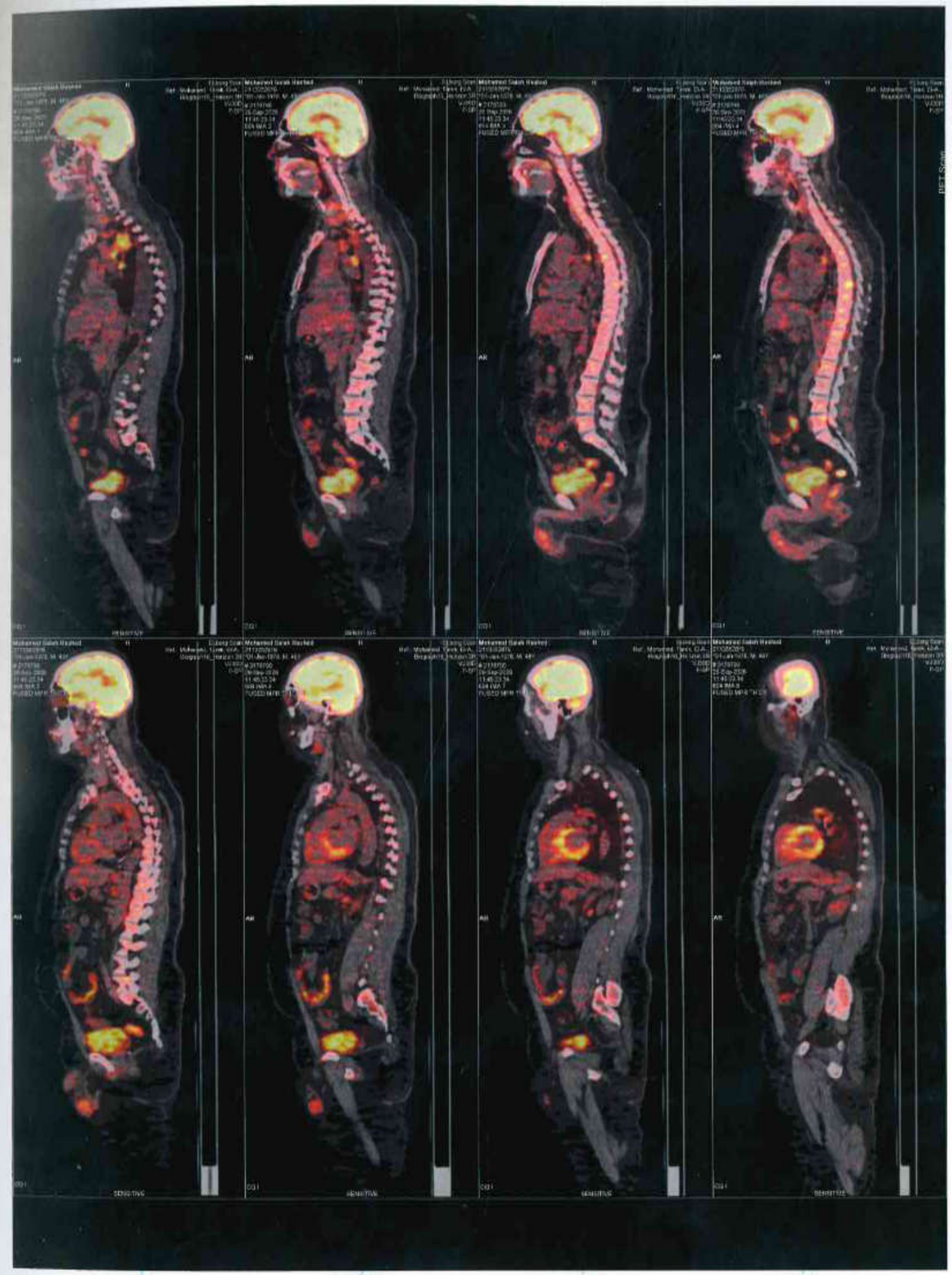

Mohamed Salah Rashed
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syngo.PET&CT Oncology 8.18.2602.601
L12VOI1 PT
Max: 2.99 SUV-bw
Mean: 1.62 SUV-bw
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Mohamed Salah Rashed
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*01-Jan-1978, M, 48Y
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Ref.: Mohamed, Tarek, El-Attar
syngo.via
syngo.PET&CT Oncology 8.18.2602.601
L14VOI1 PT
Max: 7.62 SUV-bw
Mean: 4.28 SUV-bw
10cm
F
SENSITIVE





PET Scan
[[DOCTYPE PET_SCAN]]





PET Scan

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Visit Number:	46826512338			Client Name:	
Patient Name:	Mohamed Salah Rashed			Referred by Prof:	Tarek Mohamed
Age / Sex:	48 year / Male			Exam Date:	26 Sep 2026
Patient ID:	2113352676	**Acc**	**2178790**	Printed:	27 Sep 2026 09:42:54 AM

PT

- Left renal cortical cyst.

VI) <u>The rest of PET / CT study appears normal.</u>

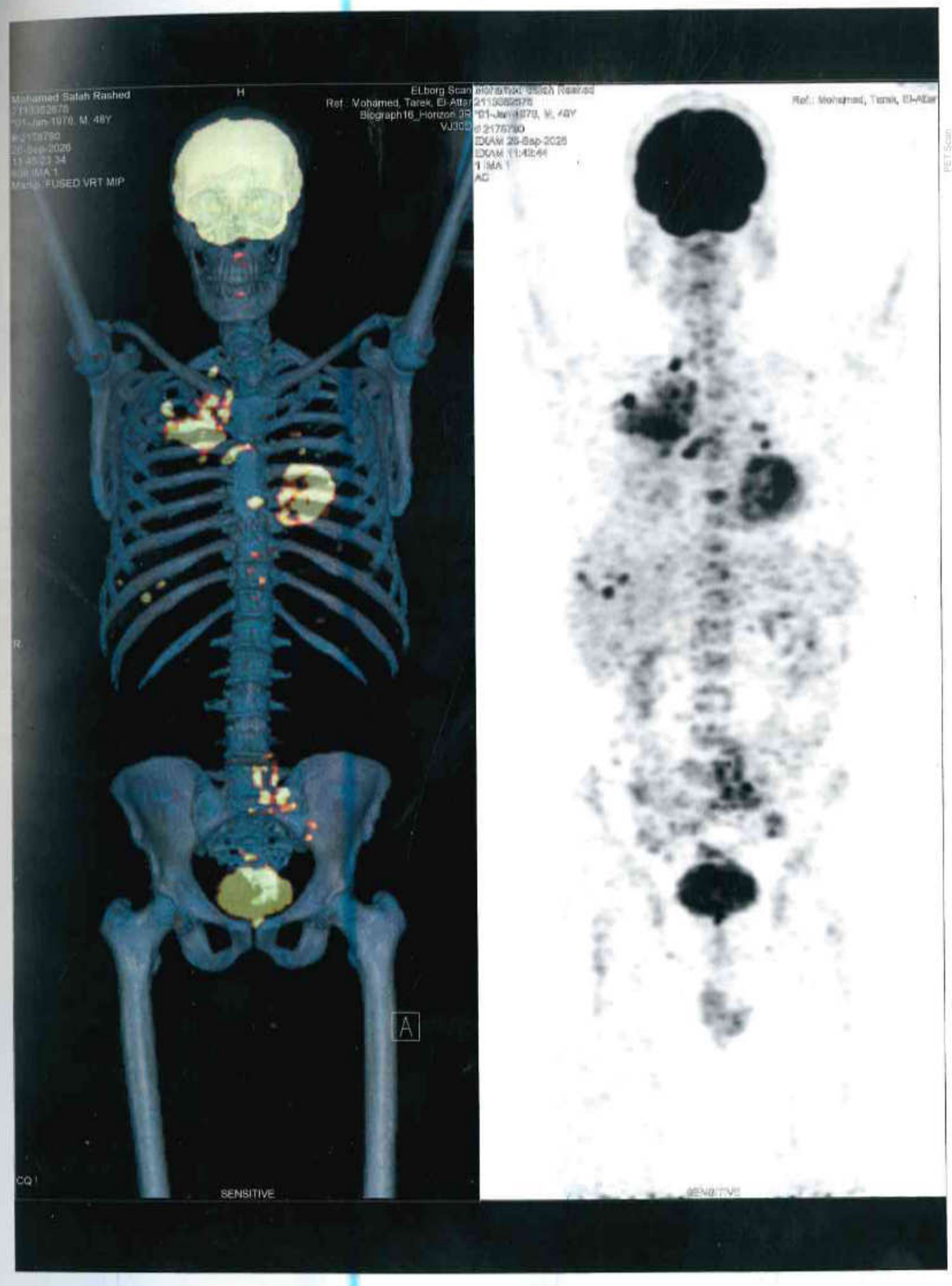
VII) <u>Normal physiological tracer biodistribution is seen over cerebral hemispheres, heart, bowel, bone marrow and urinary tract.</u>

<u>CONCLUSION:</u>

<u>PET/CT study showing:</u>

- **Metabolically active (FDG avid) right upper lung lobe consolidative mass like lesion together with diffuse bilateral pulmonary reticulo-nodular infiltrates. For histopathological correlation.**
- **Metabolically active FDG-avid multiple nodal (mediastinal/hilar, supraclavicular and upper abdominal), few hepatic and widespread osseous lesions, mostly metastatic deposits.**

Dr. Sally Abdelaziz, MSc **Prof.Khaled Aboulfotouh**



رئيس اقسام المعامل وبنك الدم

لواء طبيب / بهاء الدين عبد الفتاح جاد الدماطى

Patient ID	202653121	Patient Name	محمد صلاح راشد حسن مسعود	Sex	Male
Refering Doctor	ايمن عبدالحميد فرغلى			Age	49Y - 2D
Request Date	24/09/2026 16:15			Result Date	01/10/2026 12:19
Location	Chest clinic				

PATHOLOGY REPORT

Report Number:2837/26

Specimen

Right upper lobe lung mass, for revesion of Immunohistochemistry.

Immunohistochemical Results:

CK: Positive staining of tumor cells.
TTF-1: Positive staining of tumor cells.
Napsin A: Positive staining of tumor cells.
P63: Negative staining of tumor cells.
Synaptophysin: Negative staining of tumor cells.
Chromogranin: Negative staining of tumor cells.
Ki-67: Proliferative index 20%.

Diagnosis: Right upper lobe lung mass:
Picture is keeping with primary lung adenocarcinoma.

DR Signature
Dr. Fatma Samy.MD
A. Prof.of
Pathology,ASU.

Reviwed by : Shrouk Atef Lotfy Ibrahim

Verified by : Shrouk Atef Lotfy Ibrahim

رئيس الوحدة
أ.د. محمد أكرم نوح
أ.د. هدى إسماعيل - أ.د. نيفين طاحون - أ.د. حبيبة الفندى
د. مى جاد - د. مصطفى احمد - د. محمد الشناوى
د. انجي الحريري - د. احمد على شحاتة

فرع : B-Nasr City 3 (Al-Ahly Club)

إسم المريض : محمد صلاح راشد

محول من أد : محمود محسن

رقم الزيارة : 46826512079

40162 السن : 49

تاريخ الحضور : 17-09-2026

تاريخ الاستلام : 27-09-2026

NATURE OF SPECIMEN:

External paraffin block for immunostaining.

GROSS:

Sections immunostained using Ventana BenchMark Ultra autostainer. Ventana Ultra View DAB detection system was used.

MICROSCOPY:

CK (Pan-keratin: AE1/AE3/PCK26): Diffusely positive.
Synaptophysin: Negative.
Chromogranin: Negative.
TTF1: Positive.
Napsin A: Positive.
p63: Faint focal reaction.
Ki67: Labels about 15% of the tumor nuclei.

DIAGNOSIS:

Immunostaining: As stated above. (NB. This case was referred for immunostaining only and requested markers were determined by the referring pathologist. For final diagnosis of the case, please refer to the requesting pathologist).

Dr. Habiba Elfendy

Authentication Date: 21-09-2026

رجاء الاحتفاظ بالشرائح المرفقة كسجل دائم للحالة - لايقوم المعمل بتسليم بلوكات الشمع الا للحالات السرطانية فقط ويتم الاحتفاظ بهذه البلوكات لمدة عام بعد تشخيص الحالة

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alborglab.com
alborgscan.com

Name	محمد صلاح راشد حسن مسعود	Pt. Code	202653121	Reg.Date	06-09-2026 15:27:49
Age	48 Y	Lab. Code	-445255	Collection Date.	06-09-2026 14:51:00
Sex	Male	Location	وحدة المناظير مناظير س	Verify Date	13-09-2026 14:41:58
Ref.by		Room		Print Date	

FP-5.8-01/01

Pathology

Report Number :2837/26
Specimen :
1. Right upper lobe lung mass,bronchoscopic biopsy.
2. Broncho-alveolar lavage cytology.

Gross:
1)Multiple tiny tissue fragments, few mms in aggregates, serially sectioned and totally embedded.
2)80 cc of reddish fluid.

Mic:
1. Serial examined reveal mucosal fragments showing infiltration by neoplastic growth in form of trabeculae and cords of atypical cells surrounded by fibrovascularized stroma. The cells have miid nuclear pleomorphism.
2. Smear examined reveal bronchial cells, alveolar macrophages and few neutrophils. No malignant cells could be detected.

Diagnosis:
1. Right upper lobe lung mass; bronchoscopic biopsy:
Atypical epithelial neoplasm, for immunophenotyping (CK, synaptophysin, chromogranin, TTF1, napsin A, P63, Ki_67).

2. Broncho-alveolar lavage cytology:
Negative for malignant cells.

DR Signature
Dr. Fatma Samy.MD
A. Prof.of Pathology,ASU.

Mohamed Salah Rashed
01-JAN-1978 46826512079
46826531247 -356
Br-PathB Block
Ahaed Ragab Ramadan17-09-2026 19:18:

Reviwed By: DR/Shrouk Atef
Verified By: DR/Shrouk Atef

El-Tesseen St., Fifth Settlement - New Cairo
Tel.: 23131804 Fax: 23131801 Hot Line : 19448
شارع التسعين - التجمع الخامس - القاهرة الجديدة
سنترال : ٢٣١٣١٨٠٤ فاكس : ٢٣١٣١٨٠١ الخط الساخن : ١٩٤٤٨
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MOLECULAR PATHOLOGY REPORT
Proceed through stages

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Collection Date: 03/10/2026	**Patient Name:** الاستاذ/ محمد صلاح راشد
Review Date: 05/10/2026	**Patient No.:** 90-40681
	Accession No.: 90261011755
Referring Doctor: أ.د طارق هشام	**Age:** 49Y

NEXT GENERATION SEQUENCING (NGS)

EGFR Report2

Test methodology:

Real-TimePCR based mutation assay was done on DNA extracted from Plasma sample to screen for 29 somatic mutations in exons 18, 19, 20 and 21 of the EGFR gene using "AmoyDx® EGFR 29 Mutations Detection Kit" assays were run on the Bio-Rad CFX96 Touch Real-Time PCR detection system.

Result

EGFR: No actionable variants detected in the analyzed regions of EGFR gene.

Result interpretation:

The plasma sample tested exhibits no actionable EGFR variant in DNA extracted from Mohamed Salah. According to NCCN guidelines The Patient is unlikely to respond to EGFR TKI therapy.

Recommendations:

According to NCCN guidelines, if Liquid biopsy mutation detection is negative, tissue biopsy should be considered.

References:

*National Comprehensive Cancer Network clinical practice guidelines in oncology (NCCN Guidelines). Non-small cell lung cancer.Version 5.2026.

Pt. Branch: مصدق1

Printed by: محمود يحيى310501سلمي علي

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معامل الفا
16191

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L A B S

Collection Date: 03/10/2026	**Patient Name:** الاستاذ/ محمد صلاح راشد
Review Date: 05/10/2026	**Patient No.:** 90-40681
	Accession No.: 90261011755
Referring Doctor: أ.د طارق هشام	**Age:** 49Y

NEXT GENERATION SEQUENCING (NGS)

EGFR Report2

Additional information

The test scope includes;

ONLY 29 mutations in exons 18, 19, 20 and 21 of EGFR as follows.

- Exon 18: EGFR:c.2155G>A p.(Gly719Ser), EGFR:c.2156G>C p.(Gly719Ala) or EGFR:c.2155G>T p.(Gly719Cys)),but does not distinguish between them.

- Nineteen deletions in exon 19: (detects the presence of any of 19 deletions but does not distinguish between them).

EGFR:c.2235_2249del15 p.(Glu746_Ala750del), EGFR:c.2235_2252>AAT p.(Glu746_Thr751>Ile) ,EGFR:c.2236_2250del15 p.(Glu746_Ala750del), EGFR:c.2236_2253del18 p.Glu746_Thr751del) ,EGFR:c.2237_2251del15 p.(Glu746_Thr751>Ala), EGFR:c.2237_2254del18 p.(Glu746_Ser752>Ala) ,EGFR:c.2237_2255>T p.(Glu746_Ser752>Val), EGFR:c.2238_2255del18 p.(Glu746_Ser752>Asp) ,EGFR:c.2238_2248>GC p.(Leu747_Ala750>Pro), EGFR:c.2238_2252>GCA p.(Leu747_Thr751>Gln) ,EGFR:c.2239_2247del9 p.(Leu747_Glu749del), EGFR:c.2239_2253del15 p.(Leu747_Thr751del) ,EGFR:c.2239_2256del18 p.(Leu747_Ser752del, EGFR:c.2239_2248TTAAGAGAAG>C(complex) ,EGFR:c.2239_2258>CA p.(Leu747_Pro753>Gln), EGFR:c.2239_2251>C p.(Leu747_Thr751>Pro ,EGFR:c.2240_2251del12 p.(Leu747_Thr751>Ser), EGFR:c.2240_2254del15 p.(Leu747_Thr751del) ,EGFR:c.2240_2257del18 p.(Leu747_Pro753>Ser),but does not distinguish between them.

- Exon 20: EGFR:c.2303G>T (p.Ser768Ile), EGFR:c.2369 C>T (p.Thr790Met) and three insertions in exon 20 (detects the presence of any of 3 insertions but does not distinguish between them): EGFR:c.2319_2320insCAC p.(His773_Val774insHis), EGFR:c.2310_2311insGGT p.(Asp770_Asn771insGly), EGFR:c.2307_2308insGCCAGCGTG p.(Val769_Asp770insAla Ser Val).
- Exon 21: c.2582T>A p.(Leu861Gln), c.2573T>G p.(Leu858Arg)

Test limitations:

Results are dependent on samples being correctly labeled and information supplied being correct and complete.
Mutations where reported conform to HGVS guidelines on mutation nomenclature (www.hgvs.org) and are named according to the EGFR MANE select (NM_005228.5).

The AmoyDx® EGFR 29 Mutations Detection Kit analytical sensitivity and limit of detection are reported to allow detection of 1% mutant DNA in a background of 99% normal DNA at 10 ng sample DNA amount.
The presence of mutations at a frequency below the limit of detection of this assay is not excluded.
Chemotherapy treatment can affect ctDNA detectability (National Comprehensive Cancer Network, European Society for Medical Oncology).

Prepared by: Consultant, MD

Dr. Samar Saad

Authorized by: Head of NGS Unit

Dr. Sahar A. Sharaf

Pt. Branch: مصدق1

Printed by: محمود يحيي310501سلمي علي

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Collection Date: 07/10/2026
Review Date: 13/10/2026

Referring Doctor: أ.د طارق هاشم

Patient Name: الاستاذ/ محمد صلاح راشد
Patient No.: 90-40681
Accession No.: 4261065423
Age: 49Y

PATHOLOGY UNIT

Path. No. (15805-26).

Clinical Data & Nature of the specimen:
Right upper lobe lung mass, block, Immunostaining.

Gross Examination:
Sections autostained for ALK (D5F3) and Anti-BRAFV600E (VE1) monoclonal antibodies using Ventana BenchMark (ULTRA) and OptiView DAB detection kit.

Microscopic Examination:
Section treated for ALK (D5F3) revealed intense granular cytoplasmic staining of tumor cells.
Section treated for Anti-BRAF antibody revealed no cytoplasmic staining of tumor cells.

Diagnosis:
Right upper lobe lung mass, block, Immunostaining:-

*ALK (D5F3): Positive clinical diagnosis for ALK status.
*Anti-BRAF V600E (VE1): Negative for BRAF V600 E mutation.

Prof. Dr. Wael Shawky, MD

Reviewed By:

Pt. Branch: وحدات الرئيسي

Printed by: 39048علاء سعيد عبدالرحيم

PATHOLOGY

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